



GHK-Cu

Information sheet written for the 100 mg vial TrueHold holds in stock. Not a generic internet protocol.

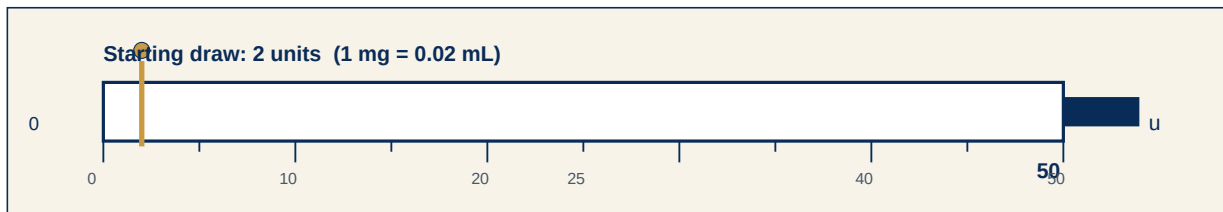
THIS VIAL — STARTING DOSE (read this first)

GHK-Cu · 100 mg lyophilized · mix with 2 mL bacteriostatic water · 50 mg/mL · 1 unit = 500 mcg

Start: draw **2 units** on a U-100 syringe (**0.02 mL = 1 mg**). On a 0.5 mL (50-unit) U-100 syringe, draw to the 2 mark.

Route: Subcutaneous (belly, love-handle, or upper arm). Solution may look faintly blue-green from copper — that is expected.

Rhythm: Once daily, 5 days on / 2 days off, for 8–12 weeks, then a break.



1. What it is — the molecule, how large it is

GHK-Cu is glycyl-L-histidyl-L-lysine bound to copper(II). The peptide alone is 340 Da; the copper complex is about 404 daltons — one of the smallest things in this catalog. Isolated from human plasma by Pickart in 1973. Plasma levels fall with age (on the order of 200 ng/mL in young adults vs ~80 ng/mL later). TrueHold stocks a 100 mg lyophilized vial.

How large is it? (daltons, log-ish bars — bigger bar = bigger molecule)

GHK-Cu	404 Da
NAD+	663 Da
SS-31	749 Da
Semax	814 Da
MOTS-c	2175 Da
Retatrutide	4731 Da
Tirzepatide	4814 Da

2. How it works in the body

The copper-peptide complex is a signaling molecule for wound and matrix remodeling: collagen, elastin, glycosaminoglycans, and antioxidant enzymes in skin models. Topical cosmetic use is the well-studied route. Injectable use is a different, thinner evidence pile. The liquid may look faintly blue-green. That is copper, not contamination, if the COA matches.

3. Testing data — animal

Wound and hair models in animals are the backbone of the regenerative story. Those papers do not equal a human injectable label.

4. Testing data — human, what is expected, hopes



Topical GHK-Cu (copper tripeptide-1) has cosmetic trials versus vitamin C / retinoic acid creams for appearance measures. There are no adequate Phase 3 trials of subcutaneous GHK-Cu. Community injectable handling is typically 1–2 mg daily, which is why this 100 mg / 2 mL vial is written so that 2 units = 1 mg and 4 units = 2 mg.

- Skin quality and tissue-remodeling notes
- Hair and wound-appearance research (mostly topical data)
- A small daily draw from a large 100 mg vial

5. Have people hurt themselves? Risks

Topical use has a long cosmetic safety record. Injectable use does not. Risks: injection-site irritation, unknown chronic copper load, and a real problem if you have Wilson disease (cannot excrete copper). At 1–2 mg the elemental copper is small compared with food, but that is not a license for unmonitored years of shots. Stop for rash, breathing trouble, abdominal pain with yellowing, or a neurologist-level tremor/change that could be copper-related.

6. FDA — approved or not

NOT FDA-APPROVED as an injectable drug. Topical copper tripeptide-1 appears in cosmetics, which are not drugs. TrueHold's 100 mg vial is a research formula, not a skin medicine.

7. Reconstitution for this exact vial

Check the label: it must say GHK-Cu and 100 mg powder in a standard ~3 mL glass vial. You need bacteriostatic water (BAC), alcohol swabs, and U-100 insulin syringes (0.5 mL / 50-unit for every starting draw on this sheet; 1 mL / 100-unit if a later step exceeds 50 units).

- Wash hands. Wipe the rubber top with an alcohol swab.
- Draw **2 mL** of BAC into a mixing syringe.
- Insert through the stopper. Inject slowly down the inside glass wall — not straight onto the powder.
- Gently roll or swirl until clear. Do not shake.
- Wipe the top again. Write today's date on the vial. Refrigerate 36–46°F. Use within 28 days.
- Mix and store in this vial only. Do not transfer.

8. Loading the 0.5 mL syringe — schedule for this vial

U-100 math is the same on a 0.5 mL or 1 mL syringe: 100 units = 1.00 mL, so 50 units = 0.50 mL (a completely full 0.5 mL syringe). Starting draws on this sheet all fit 0.5 mL. The green row is the start.

When	Units (U-100)	mg	mL	0.5 mL syringe	How often
Weeks 1–4 (start here)	2	1	0.02	Draw to 2	Once daily, 5 days on / 2 days off
Weeks 5–12 (if tolerated)	4	2	0.04	Draw to 4	Once daily, 5 days on / 2 days off

- 100 mg in 2 mL → 50 mg/mL. 1 unit = 0.50 mg. 2 units = 1.0 mg; 4 units = 2.0 mg — the commonly cited injectable research window.
- Use a 0.5 mL U-100 syringe with 1-unit marks. Draw slowly to the 2 (then 4) mark. Do not confuse this with a 50-unit fill.
- At 2 units five days per week one mixed vial lasts about 20 weeks; still cycle off after 8–12 weeks.
- Do not use if you have Wilson disease or another copper-handling disorder.

Fulfillment

- Prep and local delivery: Las Vegas residents only.
- Shipping: dry (lyophilized) vials only — we do not ship mixed liquid.
- Interest orders are confirmed by phone before payment.

Working with the team

- Share a phone number so the team can call.
- Confirm Las Vegas residency for prep and local delivery.
- Complete required documentation with the team.



- If anything on this sheet disagrees with your vial label or COA, stop and call before mixing.

Sources

- Pickart GHK-Cu plasma peptide; cosmetic copper tripeptide-1 literature
- TrueHold 100 mg / 2 mL: 2 units = 1 mg, 4 units = 2 mg
- TrueHold 100 mg vial / 2 mL BAC so 2 units = 1 mg, matching the 1–2 mg daily research-handling range.

Protocol note

TrueHold writes these sheets for the milligram vials we hold. Mix volume and unit marks were double-checked so milligrams, milliliters, and U-100 units agree. This is educational research information, not a prescription and not medical advice. The Telegram bot does not dose in chat — read this PDF. For an individualized plan, use /schedule. Research use only.

